INITIAL

TEMPLATE FOR PRIOR AUTH/PRE-DETERMINATIONS
FOR USE BY PROVIDER THROUGH REQUEST TO LANTHEUS LINK

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[Date]

ATTN: [Dept Name]
[Name of Insurance Company]
FAX #: [fax #]

RE: Prior Authorization/Pre-Determination request for PSMA-targeted PET imaging with PYLARIFY®(piflufolastat F 18) Injection

ICD10 Diagnosis Code(s):

CPT / HCPCS Code(s):

Patient Name:
Subscriber ID #:
Group #:
Date of Birth (DOB):
Pending Ref #:

Dear Medical Reviewer:

I am writing to request prior authorization/pre-determination for Prostate-Specific Membrane Antigen (PSMA) targeted PET imaging with PYLARIFY® (piflufolastat F 18) injection for my patient [patient name] who has confirmed prostate cancer with ***suspected metastasis and is a candidate for definitive therapy.***

(Please include patient details that support authorization in the paragraph below (e.g. date of prostate cancer diagnosis, current PSA and Gleason grade) and then complete this section with the provided language)

Patient has been diagnosed with prostate cancer on (date) and is a candidate for definitive therapy.

- He has a PSA of xxxx at the time of diagnosis
- Results of the biopsy showed Gleason Score of (x)
- NCCN risk category

(Refer back to payer policies for specific list of medical criteria to customize above)

Based on this assessment and current clinical guidelines, this patient has (unfavorable intermediate; high; very high risk) prostate cancer and is suspected to be at risk for metastasis prior to definitive therapy.

NCCN Clinical Practice Guidelines in Oncology – Prostate Cancer Version 4.2024 – May 17, 2024 indicates:

- F-18 piflufolastat (PYLARIFY®) can be considered as an alternative to standard imaging of bone and soft tissue for initial staging, the detection of biochemically recurrent disease, and as a workup for progression.
- Because of the increased sensitivity and specificity of PSMA-PET tracers for detecting micrometastatic disease compared to conventional imaging (eg, CT, bone scan) at both initial staging and biochemical recurrence (BCR), the Panel does not feel that conventional imaging is a necessary prerequisite to PSMA-PET and that PSMA-PET/CT or PSMA-PET MRI can serve as an equally effective, if not more effective front line imaging tool for these patients[1]

PSMA-targeted PET imaging will assist in determining if his prostate cancer has spread to other parts of the body and potentially impact his recommended treatment. The purpose of using PSMA-targeted PET imaging with piflufolastat F 18 injection is to detect and locate metastatic spread of cancer as PYLARIFY binds to the prostate specific membrane antigen protein found on the surface of over 90% of prostate cancer cells. Based on the results of PSMA-targeted PET imaging with PYLARIFY, we believe we will be able to provide a more personalized and appropriate treatment and management plan for (name of patient).

PSMA-targeted PET imaging is very useful in the initial assessment of some men with local disease, as well as in those with disease recurrence following failed prior therapies, including men whose conventional imaging fails to identify the site(s) of recurrence. Additionally, PYLARIFY was approved by the FDA based on its sensitivity, specificity, positive and negative predictive value for identifying metastatic disease at initial diagnosis versus conventional imaging.

The suboptimal diagnostic performance of conventional imaging in patients with prostate cancer often leads to ineffective under-treatment or unnecessary over-treatment. The selection of appropriate therapies is contingent upon the accurate assessment of the patient's "true" burden of disease.

It is estimated that 30-40% of patients will fail initial therapy and require additional treatment with many of these patients harboring occult metastatic disease[2,3]. Therefore, more accurate imaging at the time of diagnosis would help better identify patients likely to benefit from first-line potentially curative treatment while identifying patients with metastatic spread of disease who would no longer be candidates for curative intent therapies.

Given PSMA-targeted PET imaging with piflufolastat F 18 injection's demonstrated ability to accurately detect prostate cancer lesions, both at initial diagnosis and at recurrence of the disease after treatment failure, utilization of piflufolastat F 18 has been shown to enable and improve disease assessment for patients for patients across the spectrum of prostate cancer.[4]

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[1] NCCN Clinical Practice Guidelines in Oncology – Prostate Cancer Version 4.2024 – May 17, 2024. Available at: https://www.nccn.org/login?ReturnURL=https://www.nccn.org/professionals/physician_gls/pdf/prostate.pdf

[2] Medical News Today. How nerves fuel prostate cancer growth. Available at: https://www.medicalnewstoday.com/articles/319789

[3] Paller CJ et al. *Hematol Oncol Clin North Am.* 2013;27(6):doi:10.1016/j.hoc.2013.08.005

[4] Morris MJ, Rowe SP, Gorin MA, Saperstein L, Pouliot F, Josephson D, Wong JYC, Pantel AR, Cho SY, Gage KL, Piert M, Iagaru A, Pollard JH, Wong V, Jensen J, Lin T, Stambler N, Carroll PR, Siegel BA; CONDOR Study Group. Diagnostic Performance of ^{18}F-DCFPyL-PET/CT in Men with Biochemically Recurrent Prostate Cancer: Results from the CONDOR Phase III, Multicenter Study. Clin Cancer Res. 2021 Jul 1;27(13):3674-3682. doi: 10.1158/1078-0432.CCR-20-4573. Epub 2021 Feb 23. PMID: 33622706.

In the OSPREY study which evaluated the high-risk patient population, at least one of three image readers detected extrapelvic lesions by piflufolastat in 12.3% (33/268) of patients, potentially up staging 32 of them from clinical M0 to M1 disease.[5] In addition, 19% (51/268) of these patients were found to have nodal metastasis as detected by piflufolastat and upstaged, including 42 patients considered clinical N0 based on conventional imaging.

Approved by the FDA as of May 26, 2021, the labeled indication is as follows:

> Piflufolastat F 18 injection is a radioactive diagnostic agent indicated for positron emission tomography (PET) of prostate-specific membrane antigen (PSMA) positive lesions in men with prostate cancer:
>
> - with suspected metastasis who are candidates for initial definitive therapy.
> - with suspected recurrence based on elevated serum prostate-specific antigen (PSA) level.

For the reasons stated above, I strongly believe that PSMA-targeted PET imaging with piflufolastat F 18 injection is appropriate and required for [patient name] and should be a covered benefit.

Thank you for your time and consideration. If you have any questions and/or concerns, regarding the procedure or this patient's medical condition, please do not hesitate to contact me at the number listed below.

We look forward to [payer name]'s timely, positive response.

Sincerely,

[MD Name]
[Practice Group Name]
[Street Address]
[City/Town, State Zip]
Phone #:
Fax #:
TIN:
NPI #:

Imaging/Hospital Facility info:
[Facility Name]
[Street Address]
[City/Town, State Zip]
Phone #:
Fax #:
TIN:
NPI #:

Enclosures

[5] Pienta KJ, Gorin MA, Rowe SP, Carroll PR, Pouliot F, Probst S, Saperstein L, Preston MA, Alva AS, Patnaik A, Durack JC, Stambler N, Lin T, Jensen J, Wong V, Siegel BA, Morris MJ. A Phase 2/3 Prospective Multicenter Study of the Diagnostic Accuracy of Prostate Specific Membrane Antigen PET/CT with ^{18}F-DCFPyL in Prostate Cancer Patients (OSPREY). J Urol. 2021 Jul;206(1):52-61. doi: 10.1097/JU.0000000000001698. Epub 2021 Feb 26. PMID: 33634707.

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